

Research Statement

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I study how government policies affect economic opportunity and access to public services in historically marginalized communities. Governments try to help places like this by subsidizing employers to raise employment, by restructuring public services to deliver them at a lower fiscal cost, and by offering incentives that attract scarce professionals to relocate there. My research asks whether these programs reach the people they target, and who bears the cost when they change.

I focus on Puerto Rico, a U.S. territory of more than three million residents with persistent poverty and historically underserved communities that remain underrepresented in economic research. For example, my job market paper studies Puerto Rico's 1994-2000 health reform, which shifted a generous publicly financed healthcare system from direct public provision and universal eligibility to private management and restricted eligibility. I estimate an increase in annual all-cause mortality of 26.96 deaths per 100,000 residents (s.e. 10.32), or 3.7 percent of the pre-treatment mean. The increase is accompanied by fewer ambulatory healthcare establishments and hospitals and lower use of early prenatal care, consistent with reduced capacity and eligibility as possible mechanisms. A second dissertation chapter examines the withdrawal of Section 936, a federal corporate tax credit phased out over ten years beginning in 1996. Using synthetic difference-in-differences and data through 2010, I estimate a 16% decrease on manufacturing employment. Which leads me to believe that corporate tax credits employment benefits are not sustainable.

My ongoing and future work extends these findings to public safety, tax incentives, and physician supply. My job market paper's estimated 41.8 percent increase in homicide mortality in Puerto Rico during the 90's, which motivates a separate evaluation of an understudied security policy that took place around that time, which deployed police and National Guard troops to public housing projects. I will test whether these interventions reduced crime or increased lethal violence, and whether poor communities bore a disproportionate burden.

Building on my IRS Section 936 research, I will examine whether Puerto Rico's Act 22's tax incentives for qualifying new residents generate businesses and jobs for long-standing residents, and whether the federally supported 2021 expansion of Puerto Rico's earned income tax credit increased employment among low-income adults.

My physician research examines both geographic shortages and physician-patient matching, informed by evidence that shared socioeconomic backgrounds can improve patient adherence. With Dr. Jori Barash, I use prior diaspora settlements to predict physician inflows following Hurricane Maria in 2017 and study their health effects in Puerto Rican enclave counties, including whether Hispanic patients benefit more. A structural location-choice model will estimate the compensation needed to attract physicians to underserved areas. As a natural extension of this physician location choice model, I will also evaluate whether physician retention incentives introduced in Puerto Rico (a historically medically underserved area) under Act 14 of 2017 improved care and health for Medicare, Medicaid, and CHIP enrollees.

Across projects, I combine policy evaluation with administrative data, including county vital statistics digitized from historical reports and physician registries, to inform the design of public programs serving historically underserved communities.